

GUIDELINES

Updated S2K Guidelines on the Management of Pemphigus Vulgaris and Foliaceus Initiated by the European Academy of Dermatology and Venereology (EADV)

Abstract

Background

Pemphigus encompasses a group of life-threatening autoimmune bullous diseases characterized by blisters and erosions of the mucous membranes and skin. Before the era of immunosuppressive treatment, pemphigus was almost always fatal. Due to its rarity, only few randomized controlled therapeutic trials are available. Recently, rituximab has been approved as first-line treatment for moderate and severe pemphigus vulgaris in Europe and the United States.

Objectives

The Autoimmune Blistering Diseases Task Force of the European Academy of Dermatology and Venereology (EADV) has initiated a thorough update of the guideline for the management of patients with pemphigus.

Results

The guidelines for the management of pemphigus were updated, and the degree of consent among all task force members was included. The final version of the guideline was endorsed by the European Dermatology Forum (EDF) and several patient organizations.

Introduction

Pemphigus encompasses a group of life-threatening autoimmune bullous diseases characterized by flaccid blisters and erosions of the mucous membranes

and/or skin. The severity of the disease is based on its progressive course, which is accompanied by increased body catabolism, loss of body fluids and proteins, and secondary bacterial and viral infections that may lead to sepsis and cardiac failure. Before the advent of systemic corticosteroids, pemphigus was almost always fatal within two years of diagnosis.

Pathophysiologically, the underlying intraepithelial blister formation is caused by IgG autoantibodies against the desmosomal adhesion proteins, desmoglein 3 (Dsg3) and/or desmoglein 1 (Dsg1), on the cell surface of epidermal keratinocytes. Pemphigus is rare, with an estimated incidence of about 2 new cases per 1 million inhabitants per year in Central Europe. Two main clinical variants are recognized: pemphigus vulgaris (PV) and pemphigus foliaceus (PF).

The pathogenic role of anti-desmoglein 1 and 3 IgG has been clearly established, as injection of patients' sera or affinity-purified IgG from pemphigus sera into neonatal mice reproduces the clinical and immunopathological features of pemphigus within 24 hours. In most patients, disease activity is closely correlated with serum levels of desmoglein-reactive autoantibodies.

Due to its rarity, only a limited number of randomized controlled trials (RCTs) are available in pemphigus, most of which are constrained by small sample sizes and lack of statistically significant differences between study groups. Some studies have compared different doses of prednisone/prednisolone, intravenous corticosteroid pulses versus placebo, azathioprine versus mycophenolate mofetil, and the use of adjuvant therapies such as methotrexate, cyclosporine, cyclophosphamide, or high-dose intravenous immunoglobulins.

The combination of systemic corticosteroids (prednisone/prednisolone, 1.0–1.5 mg/kg/day) and corticosteroid-sparing immunosuppressive agents—most commonly azathioprine and mycophenolate mofetil—has traditionally been considered standard first-line therapy by most clinicians.

Over the past 15 years, more than 1,000 pemphigus patients treated with rituximab (a monoclonal antibody targeting the CD20 molecule on B lymphocytes) have been reported in the literature. Rituximab was initially used

for severe, recalcitrant, or relapsing cases, and later adopted as first-line treatment, achieving complete remission in 80–90% of patients.